MMVA
Learner resource

Minimising and Managing Violence and Aggression

Part 1 Communication and Conflict Management

Part 2 Physical Intervention Theory

Reading before the course and reflection afterwards

Clear explanations, worked examples and key reminders to help you understand the course and return to its ideas in practice.

Read at your own pace. Use the contents, glossary and quick reference to return to the topic you need. Reflection prompts are optional and do not require written answers.

A4 reading edition | Version 1.1 | 28 October 2026

Use this resource with supervised learning and current workplace procedures. Reading does not establish physical intervention competence.

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GETTING STARTED

Before and after your course

Both course PowerPoints

Learning aim Use this resource to prepare for learning, revisit explanations and connect the course to practice.

This MMVA learner resource explains the ideas behind Part 1 Communication and Conflict Management and Part 2 Physical Intervention Theory. Read it before the course to become familiar with the language and questions, then return to it afterwards when you want to understand a point more fully. The central message is to prevent distress and harm, communicate with respect and use the least restrictive lawful response.

If you are reading before the course

Begin with A safe and professional start, Preventing conflict before it grows, Making a dynamic risk assessment and Communicating so that people understand. Then read Rights and restrictive practices and Recognising medical risk during a crisis. Notice unfamiliar terms and anything you would like your trainer to explain. You do not need to complete written tasks or know the answers in advance.

If you are returning after the course

Choose the chapter connected with the part of the course you want to revisit. Read the explanation and worked example, then consider the short reflection prompts. Think about what you understood differently after the course and what you could change in your own practice. Use the final chapter on key reminders when you need a quick route back to the relevant explanation.

The worked examples are fictional learning examples, not prescribed responses for every situation. The course discussions are included as optional prompts for reflection. Activities involving a partner, a trainer-selected video or practical teaching remain trainer-led. There is no requirement to write answers, share a personal experience or disclose trauma.

The glossary explains recurring terms. Slide references let you connect the reading to the presentations without reproducing whole slides. The online edition lets you search by topic, mark chapters as read and keep an optional personal reflection note. Notes stay in this browser on this device and are not submitted to MMVA or assessed. Use fictional or anonymised examples and do not enter confidential details.

Use the resource within your role

The resource supports understanding and reflection. It does not teach physical holds, replace supervised practical learning or establish competence. The trainer will confirm your authorised McCormack system syllabus. Follow current technique risk assessments and workplace procedures, and obtain appropriate advice where you are unsure. The legal discussion focuses on England and Wales, with England-specific references identified; different frameworks apply elsewhere.

Learner resource edition 1.1, 28 October 2026. The official references support the legal and clinical clarifications carried forward from edition 1.0. Reading progress is a personal aid, not attendance, assessment or certification.

PART 1

A safe and professional start

Part 1 slides 1 to 25

Learning aim Recognise workplace violence and understand your responsibilities.

The course aims to help you recognise risk, understand behaviour, communicate effectively, defuse conflict and learn from incidents. Success includes noticing problems early, asking for help and avoiding unnecessary restriction. A situation ending without force is a positive outcome when everyone is safe.

Work-related violence includes abuse, threats and assault connected with work. It may occur away from the normal workplace, including during visits or transport. Verbal abuse and near misses matter even when nobody has a visible injury. Reporting gives the organisation information it can use to prevent a more serious event.

Prepare before learning or working

• Identify fire exits, emergency arrangements, toilets, drinking water and the agreed break times. Follow the timetable given by your trainer; the slide timetable is an example, not a promise of the current course schedule.

• Tell the trainer privately about access needs, health concerns or adjustments that may affect participation. Ask for a pause or an alternative activity if necessary.

• Listen without talking over others, respect confidentiality and discuss incidents without identifying the people involved. Nobody needs to disclose personal trauma to take part.

• Locate your workplace violence policy, lone-working arrangements, safeguarding procedure, emergency contacts and incident reporting system.

Responsibilities work together

Employers must manage reasonably foreseeable risks from violence through suitable assessment, arrangements, information and training. Workers must use the information and training provided and report unsafe situations. A duty of care requires thoughtful, competent action; it is not a general permission to restrain someone. Health and safety responsibilities are explained by HSE [1].

Conflict is a difference or tension that needs attention. It may happen within a person, between people, within a team or between groups. Conflict does not inevitably become violence. Begin by understanding the issue and the risk rather than treating disagreement itself as misconduct.

Worked example A threat after an appointment

A worker is threatened outside the building after an appointment has ended. Because the interaction arose from work, the fact that it happened outside does not make it irrelevant to workplace safety. The worker moves to a safer place and follows the local help and reporting arrangements. The later review considers the person's experience, the appointment process, information available to staff and what could prevent a similar event. The learning is to take threats seriously and look beyond the room where the work took place.

Key points to remember

• Work-related threats and near misses deserve attention.

• Know how to obtain help and use local reporting arrangements.

• Professional responsibility includes recognising your limits.

Reflect on the course: What does violence mean at work

Course discussion to revisit | Part 1 slide 9

The opening group discussion explores what workplace violence includes and why examples beyond a visible injury matter. Before the course, consider what you already recognise. Afterwards, compare that understanding with the discussion.

• What would you recognise as abuse, a threat, assault or a near miss?

• Give an example connected with work that happens outside the workplace.

• How would you report it, and what support would you expect?

PART 1

Preventing conflict before it grows

Part 1 slides 26 to 34 and 152 to 160

Learning aim Distinguish prevention, early intervention and recovery.

Prevention starts with the ordinary conditions of care and work: clear information, meaningful choices, reliable routines, enough support and an environment that people can understand. Repeated frustration is useful information about the service. Ask what the organisation can change as well as what an individual can do differently.

Stage	What it means	Examples
Primary prevention	Reduce the conditions that make distress or conflict more likely.	Clear procedures, staff training, an accessible environment, meaningful activities and person-centred support plans.
Secondary response	Notice change and respond early.	Listen, offer choices, reduce noise, address discomfort, change the lead communicator or move to a preferred space by agreement.
Crisis and recovery	Protect people from immediate harm, then restore safety and learn.	Summon appropriate help, use the least restrictive lawful response, arrange checks and support, record what happened and review the plan.

The presentations also use Green, Amber, Red and Blue planning. Green describes everyday preventive support; Amber describes early signs and helpful responses; Red describes an individual crisis plan; Blue describes recovery and support after the event. These colours

organise an agreed plan. They do not authorise a particular intervention or mean that everyone moves through a fixed sequence.



Prevention planning should include the person's own account of what helps, observations from the team, relevant clinical information and the views of people important to them where appropriate. Medication review belongs with an authorised clinician. Changes to other people's plans must preserve confidentiality and meet their individual needs.

Worked example A predictable difficult transition

A person regularly becomes distressed when an activity ends without warning. Repeating the same instruction more firmly does not address the sudden loss of something important. The team asks what the person finds difficult and agrees an accessible warning, a clear explanation of what comes next and a meaningful choice where possible. The person helps review whether this makes the transition easier. The useful question is what can change before distress develops, rather than what staff should do once the person is overwhelmed.

Key points to remember

- Look at everyday conditions, not only the crisis.
- Involve the person in deciding what helps.
- Use Green, Amber, Red and Blue as planning prompts, not authority to restrict.

Reflect on the course: Revisit primary prevention

Course discussion to revisit | Part 1 slide 34

The prevention discussion revisits a workplace incident and looks for opportunities to act earlier. Its purpose is to consider changes by the organisation, the team and the individual, rather than concentrating only on the final response.

- What could the organisation change before the situation arises?
- What could the team or individual worker do earlier?
- Write one Green action and one Amber action, and identify who will review them.

PART 1

Making a dynamic risk assessment

Part 1 slides 35 to 37 and 50 to 56

Learning aim Assess changing risk and choose a proportionate response.

A written risk assessment prepares you for foreseeable events. Dynamic risk assessment is the continuing process of checking what is happening now. New information can change the safest response: another person arrives, an exit becomes blocked, a person becomes unwell or a calming conversation begins to work. Keep reassessing rather than defending an earlier decision.

STEP BACK	ASSESS	FIND HELP	EVALUATE	RESPOND
Pause safely	Check the threat	Use support	Consider options	Then reassess

SAFER step	Questions to ask
Step back	Can I pause, create space and avoid rushing into danger?
Assess threat	What is happening? What is known, unknown or changing?
Find help	Who can help now, and how will I summon them?
Evaluate options	Could listening, a change of environment, withdrawal or another colleague reduce risk?
Respond	Choose the safest justified action, explain it where possible and reassess its effect.

Use POPS to organise your observations.

• Person: current behaviour, communication, distress, health and known preferences.

• Objects: items that present hazards and any essential equipment.

• Place: exits, access, crowding, noise, confined spaces and help.

• Situation: what led to this moment, who else is affected and what may happen next.

Describe what you observe; do not turn appearance or diagnosis into a prediction of violence.

Warning signs in the slides include changes in speech, clenched hands, rapid movement, threatening words and reduced personal space. A single sign is not proof of intent. Compare with the person's usual presentation, ask what they need and consider pain, sensory overload or illness. Threatening behaviour and a medical emergency can occur together.

Worked example New information changes the plan

At first, a worker thinks a raised voice is caused by frustration about waiting. While listening, they notice that the person is holding their side and saying they feel unwell. The assessment changes: pain or illness may need attention, and the worker seeks appropriate help rather than continuing to argue about the waiting time. Dynamic assessment means allowing new observations to change your understanding. It is an ongoing process, not a form completed once at the beginning.

Key points to remember

• Pause where safe and keep reassessing.

• Use POPS to organise observations.

• Change your response when the information changes.

Reflect on the course: Use SAFER and POPS

Optional further reflection | Part 1 slides 35 to 36 and 53

In this learning scenario, a person becomes louder while waiting, other people gather nearby and an exit is partly blocked. Use it to think through how new information affects the SAFER and POPS assessment.

• What do you notice under each POPS heading?

• What is your first safe action and what help is available?

• What change would make you withdraw or summon emergency help?

PART 1

Understanding emotion and behaviour

Part 1 slides 38 to 57

Learning aim Recognise stress responses without making assumptions about people.

Fear, frustration and anger can affect attention, speech, movement and the ability to process information. The slides describe fight or flight and an emotional scale to make this easier to discuss. People can also freeze, become quiet, withdraw or try to appease others. These are teaching descriptions, not a diagnosis or a reliable forecast of what someone will do.

When you feel threatened, you may speak faster, become more directive or stop listening. The other person may experience these changes as another threat. Notice your own breathing, posture and voice. Where safe, slow the pace, use a short pause and ask a colleague to take over if you are losing the ability to respond professionally.

Triggers and protective factors

A trigger is something that increases distress or the likelihood of escalation for that person. Immediate triggers may include pain, noise, waiting without information, embarrassment or an unexpected touch. Background factors may include poor sleep, previous trauma, hunger or earlier upsetting events. Do not assume that the same trigger affects everyone in the same way.

The slides use the term inhibitors for influences that can reduce the likelihood of an action, such as personal values, relationships, self-regulation and previous learning. In practice, strengthen safety, understanding and trusted support. Threatening punishment or relying on fear of consequences may make distress worse. A person who is overwhelmed may not be able to weigh consequences in the way you expect.

The anger ladder illustrates opportunities to intervene before harm. It is not an inevitable progression from frustration to violence. Listen for the need underneath the behaviour and look for ways to reduce pressure. Keep immediate safety and any signs of illness in view.

Worked example The same event can mean different things

An unexpected change of staff may be a small inconvenience to one person and a major source of fear to another. If you assume the person is simply objecting to a reasonable change, you may miss the loss of familiarity or an important communication need. A useful response starts with curiosity about the person's experience. You might explain who is present, what will happen and what choices remain. Notice whether your explanation helps; do not assume a calm intention has produced a reassuring effect.

Key points to remember

• Behaviour has context; seek to understand it.

• Notice your own emotional response.

• A teaching model does not predict an individual's behaviour.

Reflect on the course: Explore emotions and triggers

Course discussion to revisit | Part 1 slides 39, 45 and 51

The course explores how frustration, anger and fear can affect both the person and the worker. Reflect on the distinction between recognising an emotion and assuming what someone will do next.

• What do frustration, anger and aggression mean to your group?

• How do fear or frustration change the way you communicate?

• What are your own triggers, and what helps you stay professional?

• Identify an immediate trigger, a background factor and a helpful response for a fictional person.

PART 1

Communicating so that people understand

Part 1 slides 58 to 69 and 97 to 99

Learning aim Use words, tone and non-verbal communication consistently.

Communication is a two-way process. You form an idea, choose a way to express it, send a message and the other person interprets it. Their response helps you discover what they actually understood. A message that sounds clear to you may still be confusing to someone with different information, language, sensory needs or previous experiences.

Use plain words, one point at a time and a pace the person can follow. Check understanding without testing or embarrassing them: "I want to check that I explained that clearly. What would be helpful to go through again?" Offer a picture, written information, communication aid or interpreter when needed. A nod does not always mean agreement or understanding.

Words, tone, facial expression, gestures and position can reinforce or contradict one another. The percentage graphic in slide 63 must not be treated as a fixed formula for all communication. The useful learning point is to make the message consistent and check its meaning with the individual. Clear words remain essential, especially when explaining choices, consent, risk or emergency instructions.

A stimulus does not require a rushed response. Where safe, choose a response that keeps the conversation open. Replace "That is not my problem" with an honest explanation of what you can do. Do not promise an outcome you cannot deliver. If the person's preferred option is unavailable, explain why and offer realistic alternatives.

Worked example Checking your message

A worker says, "We will go shortly," intending to reassure someone. The person understands this to mean within a minute, while the worker expects a longer wait. When nothing happens, trust is reduced. A more useful explanation gives the information actually known, acknowledges what remains uncertain and agrees when another update will be given. Checking understanding helps identify the gap. Clear words, a respectful tone and reliable follow-through work together; no percentage rule can replace that check.

Key points to remember

• Check what the person understood.

• Make words, tone and actions consistent.

• Give honest information and follow through.

Reflect on the course: Back to back communication

Course discussion to revisit | Part 1 slides 62 and 64 to 65

The trainer-led back to back activity limits visual cues and feedback. It illustrates how assumptions can develop when the receiver cannot easily check a message. You can revisit

the learning without repeating the activity independently.

• What was difficult when only one person could speak?

• What assumptions did the speaker make?

• How would questions, visual information and feedback change the result?

Reflect on the course: The bottle of water sentence

Course discussion to revisit | Part 1 slide 98

The bottle of water sentence illustrates how tone and non-verbal cues can change the meaning someone receives. Think about how the same words may be experienced as neutral, worried or impatient.

• Try a neutral, worried and impatient version without directing hostility at a partner.

• What changed the meaning?

• How would you repair a mismatch between reassuring words and a tense tone?

PART 1

Removing barriers and checking assumptions

Part 1 slides 70 and 81 to 96

Learning aim Adapt communication and separate observation from interpretation.

A barrier can affect sending, receiving or interpreting a message. Physical and environmental barriers include noise, poor lighting, uncomfortable temperature, pain, hunger, faulty equipment and several people talking at once. Language barriers can arise through jargon, unfamiliar words or a lack of an appropriate interpreter. Fear, distress, previous experience and distraction can also make communication harder.

The physical and psychological categories in the slides are a starting point. Many barriers overlap. For example, pain can affect attention and emotion as well as hearing the message. Ask the person what makes communication easier. Make reasonable adjustments rather than expecting them to fit the way you prefer to communicate.

Barrier or need	Possible adjustment
Noise or sensory overload	Reduce competing sound and the number of speakers; offer a preferred quieter place where safe.
Processing time or unfamiliar language	Use short sentences, pause and offer accessible written or visual information.
Hearing or visual needs	Check the preferred communication method and access to aids; obtain suitable communication support.

Barrier or need	Possible adjustment
Fear or previous trauma	Explain your role and each proposed action, avoid surprise contact and offer choices.

Aggressive, passive and assertive are descriptions of communication styles used in the slides. Use them to reflect on your own behaviour, not to label a person. Being quiet, avoiding eye contact, moving repeatedly or stepping back may reflect disability, culture, fear or personal preference. These actions do not prove rudeness, dishonesty or aggression. A mental health diagnosis does not determine risk.

Worked example Quiet does not necessarily mean agreement

A person looks away and does not answer quickly. The worker initially reads this as a refusal to engage. After allowing more time and checking communication preferences, they find that the person needs information presented differently. The learning is to separate an observation, such as a pause, from an interpretation, such as rudeness. You may need to change the environment or the way the information is given before you can understand the person's decision.

Key points to remember

• Describe what you observe before interpreting it.

• Ask about communication preferences and adjustments.

• Allow time and reduce competing demands.

Reflect on the course: Communication blocks at work

Course discussion to revisit | Part 1 slide 88

The two-part discussion identifies barriers to communication and considers practical ways to remove or reduce them. The useful learning is how you would adapt communication for the individual and check the result.

• What are the main barriers in your workplace?

• For each barrier, what could you change and who needs to help?

• How will you check whether the adjustment works for the person?

Reflect on the course: Say what you see

Course discussion to revisit | Part 1 slide 70

The appearance-based discussion examines assumptions. Its purpose is to distinguish observable information from an interpretation, and to consider how that interpretation could affect your response.

• Separate an observable fact from an interpretation.

• How might an assumption about appearance affect your behaviour?

• What further information would you need before deciding there is a threat?

PART 1

Managing expectations and building trust

Part 1 slides 73 to 86 and 112 to 118

Learning aim Explain options honestly and build a workable agreement.

Conflict can arise when people expect something different from what a service provides. Waiting, unfamiliar rules, changes of plan or unexplained decisions may leave a person feeling ignored or powerless. Proactive communication means giving helpful information before frustration grows. Proactive service delivery means changing predictable flashpoints in the service itself.

Explain your role, what will happen next and when you will provide another update. If you do not know, say so and explain how you will find out. Check whether the person has understood and whether the available option meets the need they were trying to express. Explain a necessary limit respectfully and offer genuine choices within it.

Practise assertive and respectful communication

• Listen without interrupting when safe to do so. Use prompts and acknowledgement that suit the individual rather than insisting on eye contact.

• Summarise the issue in your own words and invite correction: "Have I understood that correctly?"

• Acknowledge feelings without agreeing with a threat or making promises: "I can see that this delay is upsetting. I can check the next update with you."

• Keep the problem separate from the person. Describe behaviour and impact instead of calling someone difficult or manipulative.

• Seek an outcome that addresses the underlying need and preserves dignity. A "win-win" is a workable agreement, not pressure to accept an unfair choice.

Trust grows through consistency. A reassuring conversation will be undermined if promised updates never arrive or if different staff give conflicting explanations. Record agreed adjustments and hand them over appropriately. Revisit any plan that repeatedly produces distress.

Worked example A promise that cannot be kept

A person asks for an outcome the worker cannot authorise. Saying "Yes, I will sort that" may end the conversation briefly but create a larger problem later. The worker instead explains their role, identifies what can be checked now and offers a genuine choice between available options. Respectful limits can be clear without being dismissive. A helpful outcome may involve understanding the next step and being treated fairly even when the original request cannot be met.

Key points to remember

• Explain your role and the next step.

• Offer realistic choices and truthful updates.

• Focus on the underlying need and preserve dignity.

Reflect on the course: Positive and negative service experiences

Course discussion to revisit | Part 1 slides 60 and 74

The positive and negative service examples explore how information, waiting, choice and follow-through shape a person's experience. You can reflect on an everyday service experience without naming anyone.

• Describe a positive service experience and a negative one without naming anyone.

• What did communication, waiting, choice and follow-through contribute?

• Write a truthful explanation and two realistic options for a common workplace delay.

PART 1

Defusing conflict and creating space

Part 1 slides 100 to 121

Learning aim Use the course communication models and know when to withdraw.

De-escalation aims to reduce distress and the likelihood of harm while maintaining dignity. It starts early and remains relevant during a crisis. One person should normally lead the conversation while colleagues support safety and avoid competing instructions. The lead may change if another worker has better rapport or if the original communicator becomes overwhelmed.

SPACE	How to use it
Spatial awareness	Notice movement, other people and access to help. Respect the person's preferred space.
Position	Avoid crowding or trapping the person. Keep a safe route for withdrawal.
Attitude	Stay respectful, consider your assumptions and manage your own frustration.
Communication	Listen, acknowledge the concern and offer clear information or realistic choices.
Emphasise non-aggression	Use a relaxed, balanced position and natural gestures suited to the person and the risk.

The slides also use Open PALMS:

- Position
- Attitude
- Look and listen
- Make space
- Stance

These are prompts for non-threatening communication, not a physical technique. Personal-space distances shown in the slides are illustrative. Culture, trauma, sensory needs, mobility, the environment and the person's preference matter more than memorising a universal distance.

Avoid touching unexpectedly, pointing close to a face, blocking movement or making sudden approaches. A change of stance or hand position can be misunderstood, so consider the whole interaction. Do not increase your volume or match aggression to demand attention. Use a clear, calm message and seek communication support if the person cannot hear or process it.

Contact and cover describes complementary team roles: one communicator engages while another watches the wider situation, arranges help and supports safe withdrawal. Roles must be agreed, appropriate to the setting and within training. If you cannot manage the risk safely, withdraw when possible and summon appropriate help. Withdrawal is a planned safety response, not a failure.

Worked example One conversation instead of several instructions

Several staff begin explaining a decision at the same time. The person becomes more distressed and says that nobody is listening. The team agrees who will communicate and the others reduce competing input while maintaining appropriate support. The communicator acknowledges the concern and uses a short explanation. This does not guarantee that distress will settle, so risk is still reassessed. The aim is to make understanding easier, rather than to secure silence or compliance.

Key points to remember

- Make communication easier and reduce pressure.
- Agree who will lead the conversation.
- Know when to withdraw and summon appropriate help.

Reflect on the course: Show non-aggression

Course discussion to revisit | Part 1 slide 103

The class discussion considers how a worker can signal non-aggression through communication and space. Reflect on the principles; physical practice and any trainer-directed role-play remain part of supervised learning.

- How can position, expression, voice and space communicate reassurance?

- Who will communicate and who will support safety?
- What will you say if you need to step away and get help?

PART 1

Learning from conflict and improving plans

Part 1 slides 122 to 139 and 152 to 160

Learning aim Turn an incident into practical prevention and support.

The end of an incident is the beginning of recovery. Check immediate safety, identify injury or illness, offer support and make an accurate record. The person affected, staff, witnesses and others nearby may have different needs. Do not assume that the person who appears calm has not been affected.

The slides identify possible responses such as disturbed sleep, flashbacks, withdrawal, reduced confidence, irritability or feeling unusually alert. People respond differently and reactions may change over time. Listen, offer appropriate support and encourage access to occupational health, a manager or a health professional if difficulties persist or worsen. Do not diagnose a colleague or require them to retell a distressing experience.

WHAT	WHY	LEARN	REVIEW
Establish the facts	Understand context	Agree a change	Check the result

A useful review asks what happened, what contributed, what worked, what made matters worse and what should change. Separate immediate welfare checks from a later learning review. Involve the person in ways they choose and can manage. Identify an action, an owner and a review date; otherwise a discussion may not change practice.

Update the person's support plan with useful, agreed information. Green actions should improve everyday quality of life; Amber actions should specify recognisable early changes and helpful support; Red actions should clearly describe an authorised crisis response; Blue actions should explain recovery preferences. Review restrictive practices with the aim of reducing their need, frequency and duration. Share relevant learning securely with the people who need it.

Worked example Turning a discussion into a change

After an incident, a team agrees that communication could have been better. On its own, that conclusion gives the next shift little to use. A more useful action identifies the missing information, who will provide it, how it will be made accessible and when the change will be reviewed with the person. Reflection becomes useful when it changes everyday support. Recognising what went well matters too, because the team needs to know which approaches are worth repeating.

Key points to remember

• Include the person's experience and recovery needs.

• Identify a change, an owner and a review date.

• Share useful learning securely.

Reflect on the course: Reflect and plan support

Course discussion to revisit | Part 1 slides 126, 128 and 138

The reflection discussion links a factual account of an incident to support and a specific improvement. Consider how the person's own experience would influence the review.

• What happened and what was the person experiencing?

• What did the team do well and what could be done differently?

• How will you support the person, staff and witnesses?

• What will change in the plan, who owns the action and when will it be reviewed?

• Where could you obtain support for yourself?

PART 2

Rights and restrictive practices

Part 2 slides 8 to 12, 16 to 18 and 25 to 34

Learning aim Recognise restrictions and keep the person's rights central.

A restrictive practice limits a person's freedom, movement, choice or control. Restrictions may be visible, such as holding someone, or less obvious, such as threats, routine rules or preventing access to possessions. Consider the effect on the person as well as the intention of the worker. A measure described as caring can still be restrictive.

Type	Examples to examine
Physical	Using hands or the body to prevent movement or overcome resistance.
Mechanical	Equipment used to limit movement; consider its actual purpose and effect.
Chemical	Medication used primarily to control or restrict behaviour rather than to treat an identified condition; seek authorised clinical guidance.
Environmental	Locked access, removal of items or furniture arrangements that restrict movement or choices.
Cultural restraint	Using cultural or organisational norms to pressure someone into unwanted behaviour or prevent expression of their identity, values or beliefs. Example: refusing access to a prayer mat simply because prayer does not fit the service routine. [17]
Psychological	Threats, coercion or using control over a person's circumstances to make refusal difficult.
Seclusion or segregation	Separation from other people; formal definitions and lawful authority depend on the setting.

A blanket restriction is a rule applied to everyone without individual consideration. Ask what harm is being prevented, whether the rule is lawful, who it disadvantages and what less restrictive option exists. A person's agreement or request does not make an unsafe, degrading or otherwise unlawful practice acceptable. Consent must be informed, voluntary and capable of being withdrawn where applicable.

Human rights relevant to the course include life, protection from inhuman or degrading treatment, liberty, private and family life, and non-discrimination in the enjoyment of Convention rights [4]. Equality law protects nine characteristics: age, disability, gender reassignment, marriage and civil partnership, pregnancy and maternity, race, religion or belief, sex and sexual orientation. Check applicable duties and reasonable adjustments [5].

The RRN hospital-care toolkit gives people accessible information about rights, restrictive practices and raising concerns. Use the person's preferred format and support access to advocacy or complaints routes [11].

Worked example A rule applied to everyone

A service keeps a valued item unavailable to everyone because one earlier incident involved that item. Before accepting this as necessary, staff examine who is affected, the actual risks, the individual circumstances and less restrictive alternatives. A routine can feel normal to staff while having a substantial effect on the person's freedom and trust. The example illustrates why restrictions need scrutiny; it does not provide legal approval for keeping or removing the item.

Key points to remember

• Consider the effect of a restriction, not just its name.

• Question routines applied without individual consideration.

• Rights, dignity and access to a voice remain central.

Reflect on the course: Spot an everyday restriction

Course discussion to revisit | Part 2 slide 34

The course examples include furniture blocking movement, inappropriate daytime nightwear, bed height, wheelchair straps and locked doors. Consider their actual effect and the individual circumstances, rather than accepting a familiar practice without review.

• What effect does each have on freedom and dignity?

• What individual need, lawful basis, consent process and review would be required?

• What safer, less restrictive alternative could meet the need?

Reflect on the course: Psychological restraint reflection

Course discussion to revisit | Part 2 slides 29 to 31

The trainer-selected psychological restraint animation explores how a person's choices can be restricted without physical contact. If you have not seen it, use the questions to consider the general issue; no video content is assumed.

• What pressure or restriction did you notice?

• Could the person refuse without a penalty or threat?

• What would a respectful alternative look like?

PART 2

Understanding the legal framework

Part 1 slides 11 to 23 and 137 and Part 2 slides 17 to 22 and 42 to 47

Learning aim Identify the decision, the lawful basis and the limits of any intervention.

Lawful action depends on the person, the purpose, the setting and the circumstances. A training certificate, diagnosis, staff instruction or duty of care is not by itself legal authority to restrain. Before intervening, identify the harm you are addressing and the applicable authority. Seek senior or specialist advice where the position is unclear; do not delay necessary emergency help.

The Mental Capacity Act 2005

For people aged 16 and over in England and Wales, capacity is specific to the decision and the time. Start by presuming capacity. Take practicable steps to support decision-making; an unwise decision alone does not establish incapacity. Decisions made under the Act for a person who lacks capacity must be in their best interests, with consideration of less restrictive ways to achieve the purpose [6].

The statutory assessment considers whether an impairment or disturbance of mind or brain causes inability to make the relevant decision. The functional questions concern understanding, retaining, using or weighing relevant information, and communicating a decision. Record the actual decision and support provided. Do not infer lack of capacity simply from diagnosis, appearance, distress or disagreement [7].

Section 5 gives limited protection for qualifying acts of care or treatment where reasonable steps have been taken to establish capacity and the worker reasonably believes the person lacks capacity and the act is in their best interests. It does not excuse negligence. It is not a general immunity from liability [7].

For restraint within this framework, section 6 additionally requires a reasonable belief that restraint is necessary to prevent harm to the person, and a proportionate response to the likelihood and seriousness of that harm. The Act includes threatened force against resistance and restricting movement whether or not the person resists. These provisions are not a blanket authority for deprivation of liberty [8].

Other frameworks and justification

The Mental Health Act and deprivation of liberty frameworks require their own applicable conditions and processes. Detention does not make every intervention lawful. A care plan is not a substitute for legal authority. Ask your trainer how current procedures apply to your setting, age group and role.

Section 3 of the Criminal Law Act 1967 permits reasonable force for specified purposes relating to preventing crime or lawful arrest. It is not a routine care or treatment power [9]. Any reliance on self-defence requires a case-specific assessment of an honest belief about the circumstances and reasonable force; obtain appropriate guidance on your role [16].

Explain why an action was necessary, what alternatives were considered, why the response was proportionate, how safety was monitored and why it continued for that duration. Proportionality is not a mechanical "equal or less force" formula. Document the facts known at the time and reassess as circumstances change.

Worked Example: A refusal is not a capacity assessment

A person refuses a proposed activity. The worker disagrees with the choice and believes the activity would be beneficial. Disagreement alone does not establish lack of capacity or authorise restriction. The next questions concern the particular decision, the information and support offered, and the applicable assessment and legal framework. The important distinction is between understanding why staff prefer an outcome and establishing lawful authority to override a decision. Seek setting-specific advice when this is unclear.

Key points to remember

• Identify the specific decision and applicable legal framework.

• Refusal and diagnosis do not by themselves establish incapacity.

• A course, care plan or staff instruction is not general authority to restrain.

Reflect on the course: Explain a lawful decision

Optional further reflection | Part 2 slides 17 to 22 and 42 to 45

A person refuses support that staff believe would benefit them. This scenario helps separate the worker's preferred outcome from the decision, communication support and lawful framework that need consideration.

• What specific decision needs to be made?

• What communication support and capacity assessment may be required?

• What lawful basis, alternatives and limits must be considered before any restriction?

• Who can give setting-specific advice?

PART 2

Learning from harm and lived experience

Part 2 slides 10 to 15, 25, 32, 41 and 70 to 73

Learning aim Listen to the person and challenge practices that cause harm.

The presentations introduce Olaseni Lewis, Gareth Myatt and Jimmy Mubenga, together with learning about institutional abuse and personal experiences of restriction. These are serious learning themes. Use the trainer's selected accounts and resources; this manual does not reproduce videos or determine the facts or legal findings in an individual case.

Listen for what the person was trying to communicate, whether staff recognised distress, how decisions were made and whether anyone challenged unsafe actions. Take a report of breathing difficulty seriously. A person's resistance or distress must never be treated as a reason to ignore deterioration or to punish them.

Lived experience adds information that an incident form may miss: fear, humiliation, confusion, loss of trust or the lasting effect of being controlled. Ask how staff could have made information understandable, created real choices or prevented the situation. Give the person space to describe what helped and what made things worse.

Consider whether restrictions are applied unequally. Assumptions about race, disability, age, sex, language or mental health can influence how behaviour is interpreted and how quickly force is considered. Review patterns rather than assuming that repeated restrictions on a group are inevitable. The statutory use-of-force guidance for mental health units in England addresses reducing disproportionate use and involving people in policies [10].

Safeguarding is everyone's concern. If a practice appears abusive or creates immediate danger, summon help and act within your role to protect the person. Report promptly through the safeguarding route. Preserve a factual record and escalate if the first response does not address the concern. Do not assume that a senior worker's presence makes a practice acceptable.

Worked example Hearing a different account

The incident record says the person "settled" after an intervention. Later, the person describes feeling frightened and unable to explain that they were in pain. Both the record and the review need to take that account seriously. The absence of further visible resistance does not describe the whole experience. Listening may reveal a need to change how staff communicate, recognise concerns or provide support. It can also identify a safeguarding concern that must be escalated.

Key points to remember

• Listen to what the person says happened.

• Challenge assumptions and unequal treatment.

• Raise safeguarding concerns through the appropriate route.

Reflect on the course: Listen before drawing conclusions

Course discussion to revisit | Part 2 slides 13 to 15, 32, 41 and 71 to 72

The case discussions and Calum's Story invite attention to the person's account. You can reflect on the themes without sharing your own experience or drawing conclusions about a case you have not reviewed.

• What was important to the person?

• What early opportunity for prevention or listening was missed?

• What would you challenge, and how would you raise the concern?

• What should the organisation change and how would it know the change worked?

PART 2

Planning around the individual

Part 2 slides 23 to 27 and 48 to 51

Learning aim Combine personal, environmental and organisational information.

A person-centred assessment starts with the person's wishes, strengths, communication, ordinary routines and preferred support. Ask what keeps them well and how they want staff to respond when they are becoming distressed. Involve those important to the person where appropriate and with suitable consent or other lawful arrangements.

• Identify situations associated with distress, including personal care, transitions, waiting, unexpected touch, changes of staff and access to valued items.

• Distinguish immediate triggers from background pressures. Include pain, illness, medication effects, sleep, sensory needs, substance use and previous experience where relevant.

• Describe early warning signs in observable terms and identify de-escalation strategies that the person says help.

• Record environmental risks, access to help, staffing, space, equipment and emergency medical arrangements.

• Record any authorised crisis response, the reasons for it, specific risks and monitoring requirements, release criteria and review arrangements. Past use does not prove an intervention is safe or appropriate now.

The risk of a physical intervention is influenced by the method, position, duration, exertion, the person's health and the setting. Falls, entrapment and inability to obtain help are relevant as well as injury during contact. Children, older people, disabled people, people who are

pregnant or physically unwell and anyone with additional vulnerabilities need individual consideration. A category label cannot replace assessment.

Clinical holding or supportive equipment must be considered through consent, purpose, actual restriction and the applicable clinical and legal framework. Do not describe resistance as agreement or assume that a limited hold is harmless. Follow local authorisation and reporting requirements.

Worked example A plan with information people can use

A plan says only that a person "can become agitated." This does not explain what is important to them or what helps. A more useful plan describes their usual communication, the specific situations they find difficult, early changes that may indicate distress and agreed support. It also explains how to obtain appropriate help and how the plan is reviewed. Useful detail should increase understanding and consistency without turning past behaviour into a prediction that the person will always respond in the same way.

Key points to remember

• Start with what matters to the person.

• Make early signs and helpful support specific.

• Review the plan when information or needs change.

Reflect on the course: Build an individual support plan

Optional further reflection | Part 2 slides 23 to 24 and 48 to 51

A fictional person becomes distressed during an unfamiliar journey or appointment. Consider what a useful support plan would tell staff about the person and how the plan would be reviewed with them.

• What is important to them and what communication works?

• What are their Green supports and Amber signs?

• What help, clinical information and environmental changes are needed?

• How will the person and team review the plan after an incident?

PART 2

Recognising medical risk during a crisis

Part 2 slides 48 to 60

Learning aim Recognise deterioration and prioritise medical help.

No physical restraint is risk free. A person can deteriorate in a standing, seated or ground position. Distress, exhaustion, illness, medication, intoxication and prolonged exertion can add risk. Do not assume that a person is medically safe because they are moving, shouting or able to speak.

Restraint-related breathing compromise can occur when the airway is obstructed or the body cannot breathe effectively. Avoid pressure that interferes with breathing or circulation, including pressure to the neck, chest or abdomen. Preserve communication and seek early release from restriction. Use only the current authorised practical syllabus and individual risk assessment; this manual gives no instructions for applying a hold [12].

Act on deterioration

• A report of difficulty breathing, unusual breathing effort, choking or vomiting.

• Reduced responsiveness, sudden quietness, collapse, confusion or an unexpected change in behaviour.

• A suspected head or spinal injury, severe pain or other signs of serious illness.

• Severe agitation with overheating, extreme exhaustion or signs of a rapidly changing physical state.

Severe acute behavioural disturbance is a medical emergency presentation requiring urgent clinical assessment, not a label that establishes a diagnosis or legal authority. There can be physical, psychiatric, toxicological or other causes. Early medical help, reduced stimulation and minimising prolonged exertion are priorities. Capacity and any emergency intervention must be assessed in context [13].

A person becoming still is not evidence that the intervention is working safely. Changes may indicate deterioration. Communicate concerns immediately and activate the emergency response. Do not wait for certainty or for a routine review.

Worked example A change that must not be dismissed

A distressed person becomes unexpectedly quiet and appears less responsive. A worker might be tempted to interpret this as improvement because the interaction is less noisy. The learning from the course is to notice the change itself and raise a concern urgently, rather than treating reduced responsiveness as reassurance. The person's physical condition needs attention. Keep the distinction between a reduction in visible behaviour and evidence that the person is safe.

Key points to remember

• A quieter interaction is not proof of safety.

• Take reported symptoms and changes seriously.

• Seek urgent appropriate help when deterioration is suspected.

Reflect on the course: Recognise a red flag

Optional further reflection | Part 2 slides 55 and 58 to 60

A person says they cannot breathe and then becomes quieter. This learning example asks you to recognise why a change in responsiveness requires urgent attention, rather than assuming the person has become calm.

• What makes this an urgent safety concern?

• What will you say to the team, and who will summon medical help?

• How will you avoid mistaking reduced responsiveness for calmness?

PART 2

Responding to immediate danger

Part 2 slides 52 to 61

Learning aim Summon help and protect life while working within your training.

If someone may be deteriorating, announce the concern clearly, summon emergency clinical help and remove any restriction compromising airway, breathing or circulation as safely and quickly as possible. Follow the trained emergency release and medical response procedure. Protect the person from further harm and give responders the information they need.

For an unresponsive person, call 999 without delay or activate the emergency clinical response and ensure an ambulance response where needed. Assess breathing while help is being summoned. If the person is unresponsive with abnormal breathing, treat it as cardiac arrest, start CPR and use an AED as soon as available, following current guidance and dispatcher instructions. Gasping is not normal breathing [14].

A responsive person reporting breathing difficulty still needs urgent assessment. Continue to listen, reassure and monitor while help is obtained. Give a clear handover: what happened, position and duration of any restriction, changes observed, symptoms reported, any known relevant health information and the actions already taken. Do not diagnose the cause yourself or delay emergency care to finish paperwork.

The slides discuss incidents that go to the ground. Avoid taking a person to the floor where possible. If it occurs, use only trained, authorised responses and prioritise breathing, medical review and reducing restriction. Do not automatically move someone into a seated position or attempt to lift them without considering injury and their condition. Follow current emergency and first aid procedures.

The practical trainer must show how emergency arrangements work with the specific authorised techniques. Reading these paragraphs does not establish practical competence. Ask before the session ends if you do not understand how to summon help, communicate a stop instruction or respond to deterioration.

Worked example Giving help a clear route to the person

A colleague asks for emergency help but nobody confirms that the call has been made. A clear response identifies who is making the call and what location and incident information responders need. The team follows the emergency plan and communicates changes in the person's condition. The lesson is not to assume that a request has been acted on: confirmation and a clear handover are part of an effective response. Revisit your workplace arrangements with your trainer or supervisor.

Key points to remember

• Know how to activate the emergency response.

• Confirm that help is being summoned.

• Use your current training and give a clear handover.

Reflect on the course: Check your emergency arrangements

Optional further reflection | Part 2 slides 57 to 61

Knowing how the local emergency arrangements work is part of being prepared. Before the course, note anything you need explained. Afterwards, check any remaining uncertainty with your trainer or supervisor.

• How do you summon the emergency team or ambulance, and what location information is needed?

• Where are the AED and first aid equipment?

• Who can support the person while help is summoned?

• What training or clarification do you still need?

PART 2

Working safely as a team

Part 2 slides 2 to 7, 48 to 51 and 56 to 63

Learning aim Know your role and keep monitoring, communication and release under review.

Before practical learning, complete the trainer's venue and health arrangements, discuss adjustments and understand the stop signal. Tell the trainer about pain, breathlessness, dizziness, fatigue or distress. Consent to training practice is ongoing; ask to stop whenever you need to. Use only activities introduced and supervised by the authorised trainer.

In practice, the team needs a clearly understood lead, an agreed communicator, access to appropriate help and people able to monitor the person's condition. Roles depend on the setting and syllabus. Do not improvise a technique, use a hold from memory that you have not been authorised to use, or continue simply because another team member tells you to.

Speak up immediately about any concern. Use clear, brief messages that everyone understands. Confirm that somebody has heard and acted on a request for emergency help. Support colleagues who need to change roles, but avoid unplanned transfers or changes that create additional risk. The trained team procedure governs how any physical change takes place.

Reassess whether the restriction is still necessary at every stage. Continue appropriate communication and look for the earliest safe opportunity to reduce and end the intervention. Do not use a duration threshold as a target or a guarantee of safety. End restrictions as soon as the justification no longer applies and arrange continued care and observation according to the clinical plan.

Worked example Speaking up when another worker leads

A worker notices a safety concern while a more experienced colleague is leading. Experience and seniority do not remove the need to raise the concern. The worker uses the agreed clear message, checks that it has been heard and follows the trained response. Good teamwork makes it possible to question an action without waiting for harm. It also means recognising personal limits, asking for help and changing roles through the authorised procedure.

Key points to remember

• Know who leads and how concerns are communicated.

• Work within your authorisation and competence.

• Keep necessity, safety and ending restriction under review.

Reflect on the course: Explain your role

Optional further reflection | Part 2 slides 57 and 61

The team communication discussion explores who leads, who monitors and how a concern is raised. Reflect on the verbal messages and responsibilities without attempting physical techniques from a written description.

• Who leads, communicates, monitors and calls for help?

• What words will you use to raise a concern or request a stop?

• What tells the team that an intervention must change or end?

• What are your personal limits of authorisation and competence?

PART 2

Recording reporting and candour

Part 1 slides 127 and 130 to 137 and Part 2 slides 35 to 46 and 65 to 68

Learning aim Produce an accurate account and escalate concerns through the correct route.

Make a timely factual record in the authorised system. Separate what you directly observed, what someone told you and any interpretation. Describe words and actions rather than relying on labels such as aggressive or non-compliant. Include the person's account and relevant exact words where appropriate. Record uncertainty honestly rather than filling a gap with an assumption.

• Identify the incident, date, times, location, people involved and your role.

• Describe the context, observed behaviour, communication needs, risks and attempts at prevention or de-escalation.

• Explain the reason and legal basis for any intervention, what was done, by whom, in what position and for how long.

• Record monitoring, concerns, reported symptoms, injury, clinical checks, changes and the point of release.

• Record the outcome, support, relevant notifications, the person's views, witnesses and actions for review. Use the local form for all setting-specific required fields.

Use accurate, brief and clear writing. Keep the record complete enough to understand the decision. Avoid unexplained abbreviations. Correct paper records without obscuring the original entry; follow the audit process for electronic corrections. Sign or authenticate your entry. Protect personal information and use approved systems, not activity notes in this manual.

All incidents and near misses should follow local reporting arrangements. Some work-related injuries must also be reported under RIDDOR 2013. HSE identifies deaths, specified injuries and physical injuries causing inability to carry out normal duties for more than seven days; more-than-three-day incapacity must be recorded. The responsible reporting person should check the precise criteria and deadlines [2].

In England, CQC Regulation 20 requires openness and transparency and sets additional requirements for a notifiable safety incident. These include informing and supporting the relevant person, giving a truthful account and apology, following up in writing and keeping a record. The provider must assess the correct threshold; definitions differ by provider type [15].

Worked example Description rather than a label

"The person was aggressive" gives a reviewer little information. A factual account might instead describe the words used, the movement observed, the context and the response. If the person reported pain or said that an action hurt, include that information and what happened next. Accurate recording allows someone who was not present to understand the event and the reasons for decisions. Keep the account complete enough for review while avoiding speculation and unnecessary personal detail.

Key points to remember

• Record observable facts and the person's account.

• Explain decisions, actions, monitoring and outcomes.

• Use the correct reporting and escalation routes.

Reflect on the course: The lithium error case

Course discussion to revisit | Part 2 slides 37 to 39

The slides describe a detained patient who received double the usual lithium dose for several days, developed toxicity and needed inpatient treatment before making a full recovery. The learning discussion concerns care, harm, openness and the applicable reporting threshold.

• Was the event unintended or unexpected and during a regulated activity?

• What harm and additional treatment occurred?

• Who must assess whether the applicable notifiable safety incident threshold is met?

• What immediate care, openness, support and recording are needed?

Reflect on the course: Speaking up

Course discussion to revisit | Part 2 slide 40 and Part 1 slide 135

The speaking-up discussion asks how concerns reach someone who can act, including when the first route does not resolve the issue. Think about both staff reporting and the person's access to a voice.

• How would you raise an immediate unsafe-practice or safeguarding concern?

• What route would you use if your line manager did not respond?

• How can the person or an advocate raise a concern?

• What factual information should you preserve? Do not promise absolute anonymity.

PART 2

Recovery review and continuing learning

Part 2 slides 62 to 77 and Part 1 slides 123 to 139

Learning aim Support everyone affected and use learning to reduce future restrictions.

Offer the person an accessible opportunity to talk when they are ready, with communication support or advocacy if wanted. Ask about injury, distress, what they experienced and what would help now. Explain what will happen with their concerns. Respect a decision not to talk immediately and offer another opportunity.

Support staff and witnesses as well. Separate a welfare check from a formal review or accountability process. Listening without blame does not mean ignoring unsafe practice. Staff should have a clear route to occupational health, supervision, safeguarding advice or other appropriate support. Respect confidentiality when keeping colleagues informed.

Use the review to identify conditions that can be changed, not simply to repeat that someone should behave differently. Look at communication, staffing, environment, access to meaningful activity, clinical needs and the quality of the plan. Include successful prevention and examples of good work so that helpful approaches can be repeated.

Maintaining competence requires current training, assessment, practice within authorisation and continuing professional development. Skills and guidance change. Follow the refresher and reassessment requirements specified by MMVA, your employer and the applicable programme. Do not infer a renewal interval from this manual.

Worked example Returning to learning later

Immediately after an incident, a person or worker may need quiet, medical attention or a trusted supporter before they are ready to discuss the event. A later conversation can explore what helped and what should change. Offering another opportunity to talk recognises that recovery is individual. For your own learning, revisit one point from the course with your supervisor and identify a realistic change you can review, rather than trying to change everything at once.

Key points to remember

• Support is individual and may be needed later.

• Review practice with the person where possible.

• Use supervision and continuing learning to maintain competence.

Reflect on the course: Your learning and completion record

Course discussion to revisit | Part 1 slides 138 to 139 and Part 2 slides 69 and 74 to 77

The closing discussion connects the course to future practice, supervision and continuing learning. Reflect on a small change you can follow up, and any course or workplace clarification you still need.

- What three actions will you take back to work?
- What question remains for your trainer?
- What will you practise or clarify through supervision?
- Which post-fitness, evaluation or assessment arrangements from the course still need to be checked with your trainer?

REVIEW

Revisiting the main course ideas

Part 1 slides 140 to 151

Learning aim Refresh the central ideas without treating this reading as a test.

Use the questions and explanations below as a recap after the course or a preview of the ideas you will explore. Read the explanation, then try to connect it to an example or discussion you remember. You do not need to produce written answers. These explanations support revision and do not replace the trainer's assessment.

What kind of approach is most likely to reduce conflict?

A respectful, positive and person-centred approach. It helps create understanding and offers opportunities to resolve the problem.

What does "I am all right, you are not, so what" illustrate?

The aggressive communication style in the teaching model. Use it to reflect on behaviour, not as a permanent label for a person.

Name a useful problem-solving strategy.

Active listening, empathy, identifying the cause and offering realistic choices. Explain what you would actually say or do.

Must escalation follow a fixed sequence?

No. Respond to the actual risk and change over time. The quiz's stage labels are teaching examples, not a reliable prediction.

What should you do first in a dynamic risk assessment?

Pause or step back where safe, notice what is happening and assess the threat. Do not rush into contact.

Where does a safe system of work fit in prevention?

Primary prevention. It reduces foreseeable risks before an incident develops.

What attitude should you aim to bring to a conflict situation?

Remain professional, respectful and willing to help while recognising risk and your own emotional response.

Give an example of an environmental barrier to communication.

Background noise. Reduce it where possible, check the person's needs and adapt your communication.

What do contact and cover roles mean?

One worker normally leads communication while another supports safety, the wider assessment and access to help.

Will everyone respond to trauma in the same way or immediately?

No. Responses vary and can change over time. Disturbed sleep can occur; offer support without diagnosing.

Why share good practice after an incident?

To repeat what helped, change what did not and prevent similar harm, while protecting confidentiality.

Why check on staff after conflict?

They may have been affected by the experience and need support, a break, clinical attention or follow-up.

REFERENCE

A glossary for your reading

Terms used across both course parts

Learning aim Return to the meaning of a term when you need it.

These plain-language explanations support the course. Where a term has a formal legal or clinical definition, use the applicable law, guidance and workplace procedure.

Aggression

Behaviour experienced as threatening or harmful; describe the actual words, actions and context rather than relying on the label.

Capacity

The ability to make the particular decision at the relevant time, assessed within the applicable legal framework.

Candour

Being open and transparent; specified legal and professional duties may apply when something goes wrong.

Coercion

Pressure that undermines a person's freedom to choose; it may be present without physical contact.

De-escalation

Communication and other support intended to reduce distress and the likelihood of harm.

Dynamic risk assessment

Continuing to notice, assess and respond to changing circumstances.

Empathy

Trying to understand another person's experience and showing that you are taking it seriously.

Lived experience

A person's own experience of care, support, distress or restriction and the knowledge it brings.

Near miss

An event that could have caused harm and provides information for prevention.

Person-centred support

Support shaped around the individual's needs, wishes, communication and circumstances.

POPS

Person, Objects, Place and Situation: headings to help organise observations.

Primary prevention

Action taken before an incident to reduce the conditions that make harm or distress more likely.

Proportionate response

A response justified in relation to the actual risk and applicable legal test, with less restrictive options considered.

Reasonable adjustment

A change intended to remove or reduce a disability-related disadvantage, within the applicable duties.

Restrictive practice

A practice that limits freedom, movement, choice or control; its effect matters as well as its purpose.

SAFER

Step back, Assess threat, Find help, Evaluate options and Respond, with continuing reassessment.

Safeguarding

Action to protect people from abuse or neglect and to raise concerns through appropriate arrangements.

SPACE

Spatial awareness, Position, Attitude, Communication and Emphasise non-aggression.

Trauma-informed approach

An approach that considers the possible effects of distressing experience and seeks to support safety, understanding, choice and trust.

REFERENCE

Returning to the course in practice

Both course PowerPoints

Learning aim Find the right chapter for a question you want to revisit.

Use this page as a route back into the explanations. It is a learning aid rather than an incident response procedure. In an actual emergency, follow current emergency arrangements and your training.

If you are thinking about	Return to
A recurring difficult situation	Preventing conflict before it grows and Planning around the individual.
A situation that is changing	Making a dynamic risk assessment.
A message that was misunderstood	Communicating so that people understand and Removing barriers and checking assumptions.
A disagreement about what should happen	Managing expectations and building trust.
Reducing pressure in a conversation	Defusing conflict and creating space.
A proposed restriction or refusal	Rights and restrictive practices and Understanding the legal framework.
A change in the person's condition	Recognising medical risk during a crisis and Responding to immediate danger.
An incident account or concern	Recording reporting and candour.
What to change after the course or an incident	Learning from conflict and improving plans and Recovery review and continuing learning.

A short reflection to take into supervision

Choose one idea that stayed with you. Describe the situation where it could help, what you understand now and what you would like to do differently. Identify any policy, training or advice you need before acting. Agree how you will review the change. A small, specific improvement that can be followed up is more useful than a general intention to communicate better.

REFERENCE

Further reading and course links

Both course PowerPoints

Learning aim Find the source guidance and the right course resource.

The two MMVA PowerPoints provide the course structure. The numbered references below support the legal and clinical clarifications in this resource. Guidance was checked for edition 1.0 on 28 October 2026 and retained for this reading edition. Use current workplace procedures and obtain setting-specific advice where needed.

Registration, venue risk assessment, health declarations, post-fitness declarations, evaluation and examination links in the slides are operational course resources. Ask your trainer for the current approved links. Completing a note or reading-progress tick in the online manual does not submit those forms or confirm competence. Video reflections remain trainer-led and no video access or content is assumed.

For course enquiries and current course resources, use www.mmva.co.uk.

[1] HSE employer duties and work-related violence
https://www.hse.gov.uk/violence/employer/the-law.htm

[2] HSE reporting and learning from violent incidents
https://www.hse.gov.uk/violence/employer/reporting-learning-from-incidents.htm

[3] HSE assessing risks and control measures
https://www.hse.gov.uk/violence/employer/assessing-the-risks.htm

[4] Human Rights Act 1998 Schedule 1
https://www.legislation.gov.uk/ukpga/1998/42/schedule/1

[5] GOV UK Equality Act guidance and protected characteristics
https://www.gov.uk/guidance/equality-act-2010-guidance

[6] Mental Capacity Act 2005 Section 1
https://www.legislation.gov.uk/ukpga/2005/9/section/1

[7] Mental Capacity Act 2005 Sections 2 to 5
https://www.legislation.gov.uk/ukpga/2005/9/contents

[8] Mental Capacity Act 2005 Section 6
https://www.legislation.gov.uk/ukpga/2005/9/section/6

[9] Criminal Law Act 1967 Section 3
https://www.legislation.gov.uk/ukpga/1967/58/section/3

[10] Mental Health Units Use of Force Act statutory guidance
https://www.gov.uk/government/publications/mental-health-units-use-of-force-act-2018

[11] RRN How I should be cared for in a mental health hospital toolkit
https://restraintreductionnetwork.org/resource/rrn-publishes-new-how-i-should-be-cared-for-in-a-mental-health-hospital-resources-for-people-receiving-care-in-inpatient-units/

[12] NICE NG10 recommendations on violence and aggression
https://www.nice.org.uk/guidance/ng10/chapter/recommendations

[13] Royal College of Emergency Medicine acute behavioural disturbance guidance

https://rcem.ac.uk/wp-content/uploads/2025/05/Acute_Behavioural_Disturbance_in_Emergency_Departments_May2025_V3.pdf

[14] Resuscitation Council UK adult basic life support 2025

https://www.resus.org.uk/professional-library/2025-resuscitation-guidelines/adult-basic-life-support-guidelines

[15] CQC Regulation 20 duty of candour in full

https://www.cqc.org.uk/guidance-regulation/providers/regulations-service-providers-and-managers/health-social-care-act/regulation-20/regulation-20-in-full

[16] Criminal Justice and Immigration Act 2008 Section 76

https://www.legislation.gov.uk/ukpga/2008/4/section/76

[17] RRN Cultural restraint

https://restraintreductionnetwork.org/cultural-restraint/